

What changed on July 2, and why it is on your desk

A briefing for Alberta physicians and nurse practitioners completing the DIA Medical Report (DS2444B). September 2026.

What happened

On 2 July 2026, AISH recipients were moved to the Alberta Disability Assistance Program (ADAP). The move itself cannot be appealed. A patient who wants to return to AISH must make a new application, which requires a medical report on form DS2444B.

ADAP pays \$1,740 a month, plus a \$200 transition benefit that ends 31 December 2027, and carries employment expectations that AISH does not. Many of these patients were assessed as severely handicapped years ago and are being asked to establish it again, often with conditions that have progressed since.

You are paid for this report

Form EMP11587 authorises payment for the examination and for completion of the report, as two separate fees, invoiced by you directly to the government. It must be arranged **before** the report is completed, not after. Your patient requests it and can have it sent to your office.

A nurse practitioner may complete the DS2444B. It is not restricted to physicians.

The test the report is weighed against

A severe and permanent impairment that substantially limits the ability to earn a livelihood. Every field below is read against that test.

- **Rate to the clinical picture.** Q11–Q40 and Q41–Q68 run 0 to 3. Under-rating to appear conservative weakens the file. Complete the describe box on every 2 and 3.
- **Do not average variability.** For episodic conditions write the range, not the mean: “3 on flare days, 1 between,” rather than a blended 2. A good day is not the measure; the pattern is.
- **Cumulative effect is asked for twice.** Q40 and Q68 both ask for the cumulative level of impairment. List comorbidities at Q5 even where individually sub-severe.
- **Accommodation language works against the patient.** It reads as capable with support, which is the ADAP finding rather than the AISH one. Where clinically true, state that no accommodation makes competitive employment reliable or sustainable.
- **Prognosis at Q75 is determinative.** Where clinically supported, Permanent. Undetermined tends to default the file to the lower program. For episodic conditions, quantify frequency, duration and severity.
- **Explain any non-adherence.** An unexplained “non-compliant” can sink a strong file. Cost, side effects, access, and executive dysfunction affecting medication management belong on the record.
- **Netcare is not the full record.** It holds labs, imaging, medications and hospital records, not community clinic notes. Draw also on the chart, pharmacy record and specialist correspondence.

Accuracy is the strongest position

An overstated report can be challenged, and it can cost a patient a case they truly have. Nothing here asks you to record a finding you did not make. The purpose is to put your real findings into the language the determination weighs, and no further.

The full question-by-question guide to the DS2444B, and a fillable copy of the form that opens on any phone, are free at albertadisabilitysystembreakdown.netlify.app. The official form is at open.alberta.ca. The Alberta Disability System Breakdown is a free community campaign, not a government body, and receives no government funding. General information, not legal or medical advice. Free to share and print.

DS2444B: The Medical Report, a completion guide

A clinical, question-by-question guide for the health professional completing the disability medical report, with suggested wording keyed to the eligibility test. September 2026.

Before you use any wording on these pages

Every block here is scaffolding, to be completed to the true clinical picture and no further. Delete any clause that does not fit this patient.

Accuracy is the strongest position and the one that withstands review. An overstated report can be challenged, and it can cost a patient the case they truly have.

Your signature certifies your clinical opinion. These blocks exist to put your real findings into the language the determination weighs, not to supply findings you did not make.

For the completing health professional

This is a shorthand guide to the DIA Medical Report (DS2444B). It maps each field to what the adjudication turns on, and offers suggested wording keyed to the eligibility test. The official form is at open.alberta.ca.

The test the report must speak to

AISH eligibility: a severe and permanent impairment that substantially limits the ability to earn a livelihood. Document the limitation and its permanence, not the accommodations that would let the patient work.

The accommodation trap: the single most important framing

Accommodation language reads as capable with support, which is the ADAP finding rather than the AISH one. Where clinically true, state that no accommodation renders competitive employment reliable or sustainable.

Unpredictability and inability to self-sustain are often the strongest findings. An employer can adapt a workstation; none can adapt to unpredictable incapacity.

Relationship with applicant (Q1 to Q4)

Q1: confirm Alberta registration; give discipline and registration number. Q2 to Q4: treating duration, last contact, and frequency. Longer, regular treating relationships carry evidentiary weight, so state them plainly (for example, primary provider since [yyyy], reviewed [monthly]). A specialist seeing the patient episodically should say so; it frames the scope of the opinion.

Diagnosis and medical history (Q5 to Q10)

Q5. Diagnoses, codes, onset. List all conditions, primary and secondary, with codes and onset dates. Early or congenital onset dates support permanence, so record them precisely. Include comorbidities that compound function even if individually sub-severe; the cumulative picture is what is adjudicated.

Q6. Diagnosis detail and specific symptoms. Etiology, classification, stage or grade, and the specific symptoms per diagnosis.

Suggested wording, Q6

[Condition]: [etiology / classification]. Symptoms this patient experiences: [list]. Functional consequence: [what the symptoms prevent, for example sustained concentration, reliable attendance, self-initiation of tasks]. Onset [mm-yyyy]; course [static / progressive / relapsing].

Q7. Medical history and chronology. This is the central narrative field. Tie each condition to a duration and a functional restriction, and close on permanence and impact on earning capacity.

Suggested wording, Q7

[Patient] has a [X]-year documented history of [condition(s)], first presenting [mm-yyyy], with a [stable / progressive] course despite [treatments tried]. The impairment restricts [functions]. These restrictions are [continuous / unpredictable and recurrent] and are not resolved by workplace accommodation, because [reason, for example the limiting factor is unpredictable incapacity rather than the work environment]. In my opinion the condition substantially limits this patient's ability to earn a livelihood on a reliable, sustained basis.

Q8 to Q10. Referrals, evidence, admissions. List specialist consultations and attach reports; attach diagnostics, imaging and labs; list relevant hospital admissions with dates and reasons. Attaching documents pre-empts a later request for further information.

Degree of impairment: activities of daily living (Q11 to Q40)

Rate each item to the true clinical picture: 0 none, 1 mild, 2 moderate, 3 severe or unable. Do not under-rate to appear conservative. An accurate 2 or 3, with a one-line note in the describe column, carries the file.

Note variability, and do not average it away

Adjudication weighs cumulative burden across domains. Complete the describe box wherever a rating is 2 or 3.

For episodic and fluctuating conditions, write the range rather than the mean: 3 on flare days, 1 between, not a blended 2.

The government's own guide acknowledges that recurring conditions can cumulatively amount to severe disability. A good day is not the measure; the pattern is. Averaging erases exactly the evidence that matters.

Suggested wording, Q40

Across mobility, cognition, personal care and independent living, this patient's cumulative impairment is [moderate to severe]. Key drivers: [items rated 2 to 3]. On [flare / bad] days, [patient] requires [assistance / supervision] for [tasks]. The combined effect is that [patient] cannot independently maintain the routine and reliability that sustained employment requires.

Level of impairment: mental health and IEWS (Q41 to Q68)

The same rating discipline, 0 to 3, across mood, thought and perception, cognition, behaviour, orientation, speech, somatic and safety. The fields that most often carry these files: Q50 to Q52 attention, concentration and comprehension; Q53 executive function, which is pivotal for acquired brain injury and neurodevelopmental profiles; Q57 memory, specifying long, short or working; Q60 amotivation, Q59 impulse control and Q43 emotional dysregulation, describing the functional cost in each case. Q67, suicidality, is a clinical safety field; complete it from your assessment as you would any risk documentation.

Suggested wording, Q68

Cumulative mental-health impairment is [moderate to severe], driven by [items rated 2 to 3]. [Executive dysfunction / cognitive deficits / mood instability] mean [patient] cannot reliably [initiate and sequence tasks / sustain attention / regulate under ordinary workplace demand] without continuous external structure. These deficits are [longstanding / organic] and persist [despite treatment]. They are not correctable by accommodation, as they affect the capacity to self-organise and self-sustain that any employment presupposes.

Remedial therapy (Q69 to Q74): the permanence gate

Denials often turn on the theory that more therapy might improve the condition, so it is not yet permanent. Answer these fields so the record closes that gate where it is clinically closed: treatments tried and exhausted, and why further treatment is not expected to restore work capacity. If a treatment is nominally available but inaccessible in a clinically relevant timeframe, such as a waitlist of 12 to 16 months or more, say so. It is not meaningfully available.

Q69 to Q70. Medications and their functional effect, including sedation or cognitive side effects that themselves limit work; treatment history with timeframes and outcomes.

Q71. Current plan, when initiated, review frequency, or, if options are exhausted, say so explicitly.

Suggested wording, Q71

Current management: [plan], initiated [mm-yyyy], reviewed [interval]. Available interventions have been [tried / exhausted] with [partial / no] functional gain. [Remaining options are limited to symptom management, not restoration of work capacity. / Further options are inaccessible within a clinically relevant timeframe: [waitlist / access barrier].]

Q72 to Q73. Compliance. Answer truthfully. An unexplained non-compliant can sink an otherwise strong file. Legitimate reasons belong on the record.

Suggested wording, Q72 to Q73

Adherence is limited by [cost / side-effect intolerance / access / the disability itself, for example executive dysfunction impairing medication management]. This is a consequence of the condition and circumstances, not refusal of care.

Q74. Further treatment anticipated? If it is not expected to restore the capacity for sustained work, state that plainly.

Suggested wording, Q74

No further treatment is anticipated that would be expected to restore this patient's capacity for reliable, sustained employment. Management is directed at [symptom control / stability], not functional recovery.

Prognosis (Q75): the determinative field

Four options: Temporary, Episodic, Permanent, Undetermined. Where clinically supported, Permanent, not expected to change or improve over time with treatment, is the finding the AISH determination rests on. For episodic conditions, use Episodic and quantify frequency, length, severity and total duration; unpredictable recurrence is itself a barrier to employment. Avoid Undetermined if the record supports permanence, since an ambiguous prognosis defaults the file toward the lower program.

Suggested wording, Q75

Permanent. [Condition(s)] are longstanding and not expected to improve with further treatment. Prognosis for return to reliable, sustained employment is poor. [For episodic: episodes occur [frequency], last [duration], severity [level], onset unpredictable; between episodes the patient [residual limitation].]

Additional comments (Q76): the closing synthesis

Draw the domains together into one statement of cumulative impairment, permanence, and impact on the ability to earn a livelihood.

Suggested wording, Q76

In summary, [patient] lives with [severe and permanent condition(s)] producing cumulative impairment across [physical / cognitive / mental-health] domains. The limitations are [continuous / unpredictable], not correctable by workplace accommodation, and not expected to improve with treatment. In my clinical opinion, [patient] is substantially limited in the ability to earn a livelihood on any reliable, sustained basis.

Records to draw on, and certification

Netcare is not the full record. Alberta Netcare holds labs, imaging, dispensed medications and hospital records, not community clinic office notes. Where relevant history sits outside Netcare, draw on the clinic chart, pharmacy record and specialist correspondence so the report is complete rather than partial.

Certification. Completing the report certifies that it contains your clinical findings and medical opinion. Provide name, CPSA or CRNA number, contact details, date and signature.

The Alberta Disability System Breakdown is a free community campaign, not a government body, and receives no government funding.
albertadisabilitybreakdown@outlook.com albertadisabilitysystembreakdown.netlify.app General information, not legal or medical advice. Free to share and print.

Disability Income Assistance – Medical Report

Form DS2444B · Rev. 2026-07 · Protected B (when completed)

Accessibility note: This is a fillable working copy that captures everything the official DS2444B asks. It opens and fills on any phone, tablet, or computer. It is NOT an official Government of Alberta form. The official version is available from the Government of Alberta at open.alberta.ca.

* (Mandatory) = answer required to achieve application completion.

Applicant Information

First name *

Middle name

Last name *

Last name on birth certificate (if different)

Other Preferred First name

Date of birth *

Day

Month

Year

Gender *

Male

Female

Gender diverse

Prefer not to say

Alberta Personal Health Number

Home Phone *

Other Phone (if applicable)

Relationship with Applicant

1. The DIA Medical Report must be completed by a health professional registered in Alberta. Are you a health professional registered in Alberta? *

Yes

No

If yes, please provide discipline type

Registration Number

2. How long have you been treating the applicant? *

3. When did you last treat the applicant? * (dd-mm-yyyy)

4. On average, how often do you see the applicant?

Once per week

Once every two weeks

Once every month

More than once every month

Diagnosis(es) and Medical History

5. Specify diagnosis(es) and the DIA Medical Code(s) and/or DSM V Code(s). * (Note: attach relevant medical assessments and reports. A DIA Medical Codes reference chart is provided in the official form.)

Medical condition(s)	Date of onset (mm-yyyy) *	DIA Medical or DSM Code *

6. Provide details about the diagnosis(es) (e.g. relevant etiology, classification, stage/grade/type of disease/illness). Include the specific symptoms the applicant experiences with each diagnosis.

7. Describe the medical history relevant to the condition(s)/diagnosis(es), including chronology of presenting symptoms and progression, if any. Delineate impairment, duration, and restriction associated with each condition. *

Are documents supporting the above attached?

☐ Yes ☐ No

If no, please explain

8. Has this person been referred for further medical assessment? *

☐ Yes ☐ No

If yes, list consultations and provide consultation reports.

Specialist name	Specialty	Date Last Seen	Report attached	
			☐ Yes	☐ No
			☐ Yes	☐ No
			☐ Yes	☐ No
			☐ Yes	☐ No
			☐ Yes	☐ No
			☐ Yes	☐ No
			☐ Yes	☐ No

9. Is there other supporting medical evidence for the condition(s)/diagnosis(es) (e.g. diagnostic reports, investigations, and laboratory tests)?

*

☐ Yes ☐ No

Are documents attached?

☐ Yes ☐ No

If no, please explain

10. Admission to hospital(s) or other treatment facilities relevant to the medical condition.

Admission (dd-mm-yyyy)	Reason	Discharge (dd-mm-yyyy)	Docs attached	
			☐ Yes	☐ No
			☐ Yes	☐ No
			☐ Yes	☐ No
			☐ Yes	☐ No
			☐ Yes	☐ No
			☐ Yes	☐ No
			☐ Yes	☐ No

Degree of Impairment – Activities of Daily Living

To what degree can this person complete the following Activities of Daily Living? *

Rating scale: DK = Don't know 0 = No limitation 1 = Mild (minor assistance / extra time) 2 = Moderate (needs support / some supervision) 3 = Severe (unable; done by someone else).

DK 0 1 2 3 Describe limitation, if needed

Mobility

	DK	0	1	2	3	Describe limitation, if needed
11. Transferring: in and out of bed; on and off toilet	☐	☐	☐	☐	☐	
12. Mobility: walking, getting around	☐	☐	☐	☐	☐	
13. Physical activity: ability to participate in sustained activities and physical strength commensurate with person's age	☐	☐	☐	☐	☐	
14. Sitting	☐	☐	☐	☐	☐	
15. Standing	☐	☐	☐	☐	☐	
16. Stair climbing	☐	☐	☐	☐	☐	
Cognitive Function						
17. Reading	☐	☐	☐	☐	☐	
18. Writing	☐	☐	☐	☐	☐	
19. Understanding or following simple instructions	☐	☐	☐	☐	☐	
Personal Care						
20. Bathing and self-cleaning	☐	☐	☐	☐	☐	
21. Bladder control	☐	☐	☐	☐	☐	
22. Bowel control	☐	☐	☐	☐	☐	
23. Grooming (hair, face, teeth, hands and nails)	☐	☐	☐	☐	☐	
24. Dressing (including buttons, clasps, zips, shoelaces)	☐	☐	☐	☐	☐	
25. Select clothes for weather and situation	☐	☐	☐	☐	☐	
Independent Living Skills						
26. Meal Preparation	☐	☐	☐	☐	☐	
27. Eating: using utensils	☐	☐	☐	☐	☐	
28. Shopping for groceries	☐	☐	☐	☐	☐	
29. Housekeeping	☐	☐	☐	☐	☐	
30. Laundry	☐	☐	☐	☐	☐	
31. Transportation: ability to use available means of transportation	☐	☐	☐	☐	☐	
Social Function						
32. Attending medical appointments	☐	☐	☐	☐	☐	
33. Managing finances: ability to manage own money	☐	☐	☐	☐	☐	
34. Managing prescription medication (if applicable)	☐	☐	☐	☐	☐	
35. Managing Over-the-Counter medication (if applicable)	☐	☐	☐	☐	☐	

	DK	0	1	2	3	Describe limitation, if needed
36. Communication using phone, text, email, etc.	☐	☐	☐	☐	☐	
37. Safety: ability to maintain personal safety	☐	☐	☐	☐	☐	
38. Social interactions (get along with others, maintains social boundaries)	☐	☐	☐	☐	☐	
39. Hobbies/taking part in activities for relaxation or pleasure	☐	☐	☐	☐	☐	

40. Using the ratings of the Activities of Daily Living, describe symptoms impacting the cumulative level of an applicant's impairment: *

Level of Impairment – Mental Health (IEWS)

Level of impairment due to mental health aspects. * The checklist below consists of features/symptoms from the Intellectual and Emotional Wellness Scale (IEWS).

Rating scale: DK = Don't know 0 = Not present 1 = Mild (sometimes) 2 = Moderate (often) 3 = Severe (a barrier to well-being).

	DK	0	1	2	3	Describe limitation, if needed
Mental and Emotional States						
41. Depressive mood	☐	☐	☐	☐	☐	
42. Euphoria/Elation (elevated mood)	☐	☐	☐	☐	☐	
43. Emotional dysregulation	☐	☐	☐	☐	☐	
44. Anxiety	☐	☐	☐	☐	☐	
Thought and Perception						
45. Delusions	☐	☐	☐	☐	☐	
46. Hallucinations	☐	☐	☐	☐	☐	
47. Thought disorganization	☐	☐	☐	☐	☐	
48. Grandiosity	☐	☐	☐	☐	☐	
49. Dissociative symptoms	☐	☐	☐	☐	☐	
☐ Increase ☐ Decrease						
Cognitive Function						
50. Attention deficit	☐	☐	☐	☐	☐	
51. Concentration deficit	☐	☐	☐	☐	☐	
52. Comprehension deficit	☐	☐	☐	☐	☐	
53. Executive function deficits (e.g., self-regulation, planning and organization)	☐	☐	☐	☐	☐	
54. Insight deficit	☐	☐	☐	☐	☐	
55. Judgement deficit	☐	☐	☐	☐	☐	

	DK	0	1	2	3	Describe limitation, if needed
56. Learning deficits (specify)	☐	☐	☐	☐	☐	
57. Memory deficit	☐	☐	☐	☐	☐	
☐ Long term memory ☐ Short term memory ☐ Working memory						
Behaviour						
58. Disinhibition	☐	☐	☐	☐	☐	
59. Impulse control deficit	☐	☐	☐	☐	☐	
60. Amotivation	☐	☐	☐	☐	☐	
61. Withdrawn	☐	☐	☐	☐	☐	
Orientation						
62. Disorientation (person, place or time)	☐	☐	☐	☐	☐	
Speech						
63. Speech deficit (not due to language barrier) (specify)	☐	☐	☐	☐	☐	
Physical and Somatic						
64. Appetite change	☐	☐	☐	☐	☐	
☐ Increase ☐ Decrease						
65. Energy change	☐	☐	☐	☐	☐	
☐ Increase ☐ Decrease						
66. Sleep dysfunction	☐	☐	☐	☐	☐	
☐ Difficulty sleeping ☐ Excessive sleeping						
Safety						
67. Suicidality	☐	☐	☐	☐	☐	
☐ Ideation/Thoughts ☐ Plans ☐ Attempts						

68. Using the ratings of the Intellectual and Emotional Wellness Scale, describe symptoms impacting the cumulative level of an applicant's impairment: *

Remedial Therapy

69. Complete the chart below or attach a list of relevant medication(s). *

☐ List attached

Type of Medication	Effect on Functioning

70. Describe treatment history and results inclusive of the timeframe. *

71. Describe the current medical plan. Include when treatment was initiated, anticipated results, and how often the plan is reviewed. If no treatment/remedial approaches have been tried or are planned, explain why. *

72. Is the applicant compliant with the treatment plan recommended by their primary health professional? *

☐ Yes ☐ No

If no, please explain

73. Is the applicant compliant with a treatment plan recommended by a health professional who is not their primary health professional? *

☐ Yes ☐ No

If no, please explain

74. If further treatment is anticipated, describe the treatment and include anticipated results and estimated timeframe. *

Prognosis

75. Duration of the medical condition(s) is likely to be:

☐ Temporary – will improve over time with further treatment.

Estimated duration?

☐ Episodic – episodes recurring as follows:

(Indicate frequency, length of episodes, severity of episodes, and total duration of illness if known.)

☐ Permanent – not expected to change or improve over time with treatment.

☐ Undetermined – unclear whether it will improve over time with further treatment.

Explain

Additional Comments / Information

76. Please include any additional information relevant to the applicant's condition that the ministry should consider in determining eligibility.

Certification

The DIA Medical Report must be completed by a health professional registered in Alberta.

- ☐ I have completed and/or approved the information submitted in this report.
- ☐ This report (and attached documents) contains medical reports, clinical findings and my medical opinion at this time.

Health Professional's name (please print)		Office Address	
CPSA or CRNA registration #	Phone	City/town	
Province	Postal code	Date (dd-mm-yyyy)	
Health Professional's signature			